Monday

If anyone has questions put them in BLUE - we'd try to answer if u confused on something :)

Monday morning

1. Px comes in with +5.00/-2.00x180 R and +4.00/-3.00x90 L. They want contact lenses. What do you do?
    a. What did the Rx convert to? tyyy
    +5.25/-2.25x180 +4.25/-3.25x90 (remember to split it into two meridians!!)
    4-6D: change by 0.25D
    6-8D change by 0.50D
    8-10D change by 0.75D
    10-11D change by 1.00D and follow this trend onwards
    b. Do you get to use a calculator?? No - no calcs allowed
    c. I got 25 axis for mine and i said i would change to 20 for my lens compensating for NASAL TORQUE - then lily smiled
    d. Nasal Torque?? loll blinking action makes the lenses rotate nasally - yes, ur lens normally rotates 5-10 degrees NASALLY on blink, this is normal (hope that makes sense) yes thank uuuu
    e. If the axis is 180 tho do we choose 175 then? NO!!!! - keep 180! Only compensate for nasal torque if u have a 5 degree increment (only compensate for nasal torque if u need to adjust for cyl, e.g., if u have 175 and the lens doesnt come in 175 u need to chose 170 or 180, then u compensate for nasal torque) - hope this makes sense, dont need to do if ur lens already comes in that axis
2. (went into history taking) → Age 35, motivation, occupation
3. Why do you need to ask about their occupation? Anything else?
    - Dusty environment e.g. construction - DDs better?
4. What else do you need to ask? → Injuries, surgeries, infections. Why do you need to ask?
5. What else? → allergies. Why?
    - Should not fit if active allergic conjunctivitis
    - Also atopy → increased risk of hypersensitivity CLPC
6. What do you want to do next? → went into CL trial fitting Biofinity torics
7. What lens are you picking? What CL power
8. Why did you adjust CL power? → vertex distance - you have to correct for sph AND cyl
9. Tell me more about biofinity torics → SiH, continuous wear, BC, Dk/t. What else? Modulus
10. What is modulus? When would you pick high modulus vs low modulus?
    - High mod: easier to handle for inexperienced wearers
    - Low mod: better if complications i.e. CLPC, CLARE
11. What is Dk/t? Why is it important?
    - Dk: permeability
    - Dk/t: transmissibility, for -3 lens
12. What are signs of corneal hypoxia?
    - Epithelium:

        - Epithelial microcysts - accumulations of cellular debris or necrotic material
            - Reverse illumination
            - Mx: refit with higher Dk/t
            - Rebound response - gets worse before gets better
            - Transient
        - Vacuoles - gas-filled, smaller than microcysts, not reversed illumination
        - Epithelial thickness - long-term thinning
    - Stroma
        - Oedema - haze, striae, folds
            - Cause: anaerobic metabolism → lactic acid → osmotic gradient → fluid in stroma
            - Striae - when swelling >5%
            - Folds - when swelling >8%
                - +1% swelling = 1 fold
        - Stromal thickness - thinning if chronic oedema
    - Endothelium
        - Blebs - dark holes in mosaic
            - Transient
            - Cause - acidification of post stroma from anaerobic metabolism
        - Polymegathism/ pleomorphism - on specular reflection
            - Cause - age or hypoxic stromal acidification
    - Neovascularisation - BVs encroach from limbus onto cornea >1mm
        - Cause: hypoxia at limbus → inflammation / inflam mediators → VEGF
13. The px tells you they want to swim in their CLs, what do you tell them? → goggles. What else?
    - I got asked this Q too. She wants you to consider giving a daily disposable even if vision can't be optimally corrected. Just prescribing the max cyl possible and then give the spherical equivalent for remaining Rx. This will still give functional vision in this case.
    - Ask Px to discard lenses after swimming. Also wait 15-20 min or rinse with saline before removal or will rip off epithelium (hypotonic environment → CL sticks to cornea)

Earlier on monday (I got asked very similar to above and almost identical case) - additional q's:

- The px tells you they have worn CL's before. What do you do with this information?
    - Ask what brand, power, modality, any adverse events, why they stopped
- What sports would the px have to participate in to affect your lens choice?
    - Swimming / water sports - DDs or Rx goggles?
- What if this patient (wearing bioinfinity toric **MONTHLY** lenses) wants to **swim**?
    ABSOLUTELY NO CONTACTS BECAUSE MK RISK WITH MONTHLYS -> NEEDS RX GOGGLES OR NOTHING!!!!
- Do all CL disinfectants **not** work against acanthamoeba?

Optifree puremoist (MPS) was shown to be somewhat effective in killing the **trophozoite** form of acanthamoeba **but not the cyst form!** This is why monthly CL wearers still cannot swim in CL's!

- Why do we not do heat disinfectant?
  - Heat CL in saline for 10min at 80 deg
  - Cons:
    - Increases lens ageing, denatures protein deposits so they get stuck on lens, causes lens discolouration if preservatives in saline, HWC lenses too fragile, not portable

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Monday Arvo (IM COOKED she went into RGPs ;-;)

- Same script above and asked what lens i would give
  - Said biofinity/ biofinityxr → then she said why not give rpgs ;-;
  - Said its definitely a possibility but i would need more information
- What other information do you need?
  - topo → then she asked why?
    - Corneal toricity and eccentricity and BC readings
- She followed with a, "is topo useful in SCL fittings"
  - I said why not?
  - She followed up with a why?
  - I said it gives us measurements of the corneal curvature but not usually used for SCL tho
- Well it is useful, what else do you need to know on topo
  - *I dunno - help*
  - Maybe HVID
  - Pupil size? Topo shows pupil size???? Lmao didnt know that - I think it shows but maybe not to say it as idk if that's dim light or bright light pupil diameter (I not sure lollll))) lmao, thanks, she kept saying what else → i legit named everything like K readings, eccentricities, toricity etc
- What is a likely rigid you would recommend to this px?
  - I said need more information pls!!! Given it is rare for refractive astig to be 1.2-1.4x corneal astig, i would give SPE if refractive and corneal cyl is equal, if not CPE
- Is there an advantage of SPE over BST
  - BST is a torodial back so it can cause unwanted induced cyl
  - SPE has a +ve front surface and a torodial back so it can correct for the induced cyl and the lens is free to rotate/ does not affect vision
- How do toric RGP lenses stabilise
  - Mandell moore fitting criteria based on corneal toricity gives better tear lens exchange + lens stability
  - FST: Prism Ballast - 1.5-2pd BD <3 forgot about this, but she seemed happy with the mandell moore

    - I think bitoric does not need prism?? Bc the curves match the cornea?? Bitoric does need prims if it is a CPE so that the front surface corrects both the residual and induced astigmatism.
- Complications of hypoxia (same as above)
    - But went more into endothelial pump: why is damage to the endothelium a cause for concern
        - Endothelial cells dont regenerate → active flow of N+/K+ and ATPase ions which causes osmotic gradient → this allows passive fluid to flow out (dmg = pump no work = oedmea = bad vision)
- I forgot this too - she talked about swimming (same as above) then started talking about disinfecting solutions
    - Can you disinfect acanth blah blah → same as above
    - Then she asked what specific disinfecting agents are in disinfecting solutions
        - I said H202 has H202 3% lmao
        - Optifree is a good one cause it has aldox (tropozoite coverage), polyquad (antimicrobial)
    - Then she asked what other solutions are there: biotrue, boston etc, and she kept asking what else OTL
        - Biotrue = polyquad and biguanide
        - Renufresh = biguanide
- Ill add in how i did the spherical equiv for swimming since everyone seems to be getting confused with it
    - +5.00/-2.00x25 +4.00/-3.00x90
    - Converted to +5.25/-2.25x20 (nasal torque) +4.25/-3.25x90
    - Told lily for the LEFT EYE i would give +4.25/-2.25x90, adv px this would be more blurry but they dont need good VA for swimming
        - Lily said good but what can you do to improve VA
    - Ill do sph equiv. Since DD only comes in -2.25 cyl, ur missing out on -1.00D cyl → this coverts to -0.50 sphere
        - FINAL LEFT CL rx is +3.75/-2.25x90 this would compensate for the lost cyl, however they are over minused and may get HA so advise px this is only for swimming and NOTHING else (lily said good and smiled so im assuming??? Its correct)
- She also came bk to the SCL and asked “why did you chose a SiHy”
    - High modulus = easy handling
    - High Dk/t = more comfort = better ocular health
    - H → oxygen permeability reliant on water VS SiH → oxygen permeability not reliant on water but on silicone; water content can be maintained low for greater relief of dry eye but still high Dk/t (loose polymers blah blah blah LOL) <3

        What else? What else? What else? (kill me)-----SAMEEEE!!

Then she was like “this is all we have time for” as i awkwardly said bye

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Monday arvo :((me almost asking the same questions below, SHE KEPT SAYING: WHAT ELSE???? I THINK IM SCREWED

Spectacle Rx: R: -5.00/-2.00x25 L:-4.00/-3.00x90

L: px wants contact lenses, what do you need to ask?

- history and symptoms, have they worn CLs before? What's the occupation? What do they want to use CL for? Any history of injuries or surgeries to the eye? Any allergies?

L: you asked if they have worn CLs before, WHY do you need to ask that?

- I want to know if they had any brand preference, any complications in the past, how was their experience?

L: WHY?

- If they had any brand preference, that could be my starting point. If they had any complications, I want to make sure it's fully healed and cornea is healthy before i fit another CLs

L: You asked abt injuries and surgeries. WHY do you need to ask that?

- ....said similar things to above…

L: You asked about hobbies, apart from swimming and sports, any other hobby you wanna ask?

- If the px required high precision vision in some sports, i want to correct the cyl fully to make sure she has high clarity vision
- Near vision tasks- reading, fishing, golf

L: WHAT ELSE

- I can't think of anything else. (maybe general health? Medications? Also not sure if you asked for how many hours/day, days/week)---ye true,,ur so right me screwed i think you also need to ask about environmental factors like if its for office use, any aircon etc or if working in like dusty environment . Also ask about VDU (important for pre-presbyopes/presbyopes/myopes
- For general health:
    - Diabetes - increased risk of infection bc of neuropathy and weak attachment of epithelial basement membrane
    - Graves - exposure keratopathy
    - Medications that predispose you to dry eye; antipsychotics, antihistamines, gastrointestinal medication, diuretics, beta blockers etc.

L: WHAT ELSE do you need to ask for the history section?

- Age. and i said i can't think of anything else…:((

L: ok, now what do you do?

- I will pick a lens and start the trial –i think maybe i should have said slit lamp exam?? If that what she wanted idek

L: what lens would you pick?

- Considering the high cyl, i would pick Bioinfinity toric XR—but this is wrong, RE should be Bioinfiniy toric…no XR…:((

L: what are the power range of XR?

- i answered this wrong, but the correct answer is +20 to -20 i think
- Cyl up to 5.75DC with 5 deg axis increments

L: Ok, what other parameter do you know about XR? (think she mightve wanted u to go into cyl ranges here)--i answered about the cyl in last q

- Dk/t of 116, modulus of around 0.75, SiH material comfilcon A

L: why is SiH material better for her?

- SiH generally has higher Dk/t, good for frequent use and would avoid signs of hypoxia more

L: why is higher Dk/t important for her?

- Long time use, hence could avoid hypoxia…
- I had the same case but when she asked this question we went down the hole of corneal metabolism and the importance of having a high Dk/t since CL act as a barrier preventing oxygen to reach the corneal surface for energy pathway purposes. Soft CL are also larger so basically no tear exchange pump like RGPs do meaning having a high Dk/t is even more important to allow oxygen to reach the cornea etc

L: WHAT ELSE? WHAT ABOUT THE SCRIPT?

- This is a high script, hence could be thicker, hence Dk/t is even more important might need to note high sph and cyl makes the lens thicker and toric lenses have lower Dk/t than their spherical counterparts

L: WHAT ELSE???

- And i just died like that:((and the time is up and we finished—-basically the whole session felt like i didin't give her the answer she wanted and she was not moving on at all…--GOOD LUCK GUYS!!

Monday arvo :/

Specs R: +5.00/-2.00 x25 L: +4.00/-3.00x90 Damn everyone getting SCL whyd she turn mine into RGPs ;-; dont worry the RGP stuff is coming … oh no…. Haha rgps are a horror story yall im so scared im shaking in thmy boots

- What things do you want to know ? I said age, hobbies, how often do they want to wear, previous infection, Systemic conditions, medications
- Why do you want to know about hobbies? I started saying if theyre swimmers then they cant swim with CLS or if theyre doing high intensity sport or dirt sports
- What do you want to do next? VA and slit lamp examination
- What do you want to look for in the SLE?sign of corneal irregularity, dry eye, active corneal infection
- What lens prescription would you give? R: +5.25/-2.25x25 (told her i would do axis 20 to account for nasal rotation) L +4.25/-3.25x90
- What lens? RE Biofinity toric LE Biofinity XR Toric
- What if they want to swim with the lenses?i told her ideally would not give a contact lens for swimming and would suggest prescription goggles but since this is a CL exam i will give a daily and tell them LE will not be as clear. Tell them to wear tight fitting goggles and wait 15 minutes before removing them after swimming
- What daily lenses? I said MyDay then told her the script. RIP i told her i would give spherical equivalent for the LE and completely messed up that calculation (i have this explained in third set of questions above if anyone needs, ill make it purple so easy to find)

- Why cant they swim with the monthly? Because acanthamoeba is scary. And if theyre swimming weekly then that's not very economical isnt it
- Why cant i just clean it? I said its controversial if the cleaning solutions actually kill acanathamoeba especially in the cystic phase, so it's best to not risk it
- Why are RGPs contraindicated? I said theyre not- i just need more information
- What information? Topography and K values, eccentricity, i need to know corneal astigmatism
- Why do you need to know corneal astigmatism? I said it will affect my lens choice and briefly spoke about the different RGP designs and the relationship between refractive and corneal astig (major tangent here)
-

RGP Lens Type	Prescription and Corneal K indications
Spherical	- Low refractive astigmatism - Low corneal astigmatism (reports of up to 3.00 D, but <2.00 D is usually upper limit)
Front Surface Toric	-Usually low corneal cylinder but high refractive cylinder - For the most part, can use SCL torics rather than FST
Back Surface Toric	-At least 2D of corneal astigmatism - Refractive Astigmatism 20-40% higher than corneal astigmatism
SPE Bitoric	-At least 2D of corneal astigmatism - Refractive Astigmatism ≈ Corneal Astigmatism
CPE Bitoric	-At least 2D of corneal astigmatism - Refractive astigmatism ≠ corneal astigmatism

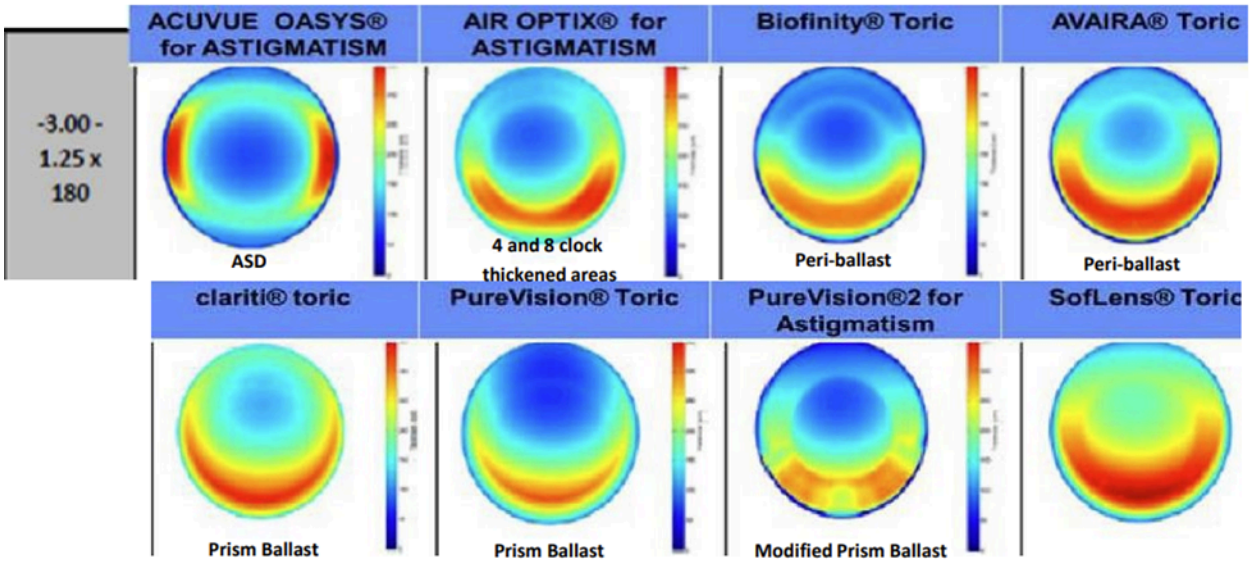
- What issues might you have with this script? I said high plus so might be low riding
- She said how do you fix this? I said increase lens diameter and/OR decrease BOZR to make a steeper lens
- Can he wear RGP and swim? I said no because of atmospheric pressure and it might suction on the eye more (i have no idea if this is correct)
    - I think there is a risk of it falling off - yes it can fall out and u cant find it, vinod talked about this in pracs

Then she said thats all xoxoxoxo I DONT KNOW WHY SHE ASKED ME SO MUCH

Rx R -5.00/-2.00x45 L -4.00/-3.00x90
Interested in CL, what would you do

- History, general health, allergies
- She probed on general health = why do u need to know/what kind of questions would you ask
- > I talked about immunocompromised increasing infection risks, diabetes = corneal desensitivity = could present to eye care later if problems arise
- If a diabetic came in wanting CL what would you do
- > Not an absolute contraindication but due to increased risk, I would review more frequently compared to a healthy individual
- What lens would i recommend = Biofinity toric in OD axis 40, Biofinity toric XR
- She asked me why not a DD? I said i dont think -2.75 cyl exists in a daily disposable for the left eye so went with monthly

    - PSA J&J Acuvue Oasys Fortnightly has 2.75DC for minus powers only (but this is not a DD??) - ye, no DD, no defs not HAHAH
    - B and L have BioTrue for astigmatism daily. Goes to -2.75 cyl, but is a hydrogel lens 😮
    -
- She asked me - tell me more about biofinity = i said modified prism ballast (wide band) design, SiHy, high dk/t, stiffer lens, higher modulus biofinity is peri-ballast?? Idk the website calls it a “optimised prism ballast” so thats what i said technically u aint wrong, periballast is a type of prism ballast lmao



Sulley, Anna, et al. *Contact Lens and Anterior Eye* (2015)

- From our lectures ^^

- > She asked stiffer compared to what? I said hydrogels, she asked compared to other SiHy is it comparatively a high modulus or low modulus Biofinity is 0.75, on the higher end - yes MyDay is lower i think 0.4? Ye myday is lowest modulus of all SiHy
- Patient wants to go swimming = I said this is a monthly lens I would NOT go swimming
- I talked about acanthamoeba
- She said what if patient wants to go to the beach or water sport = i said those recreational activities don’t need good vision, bring glasses to the beach
- What if i clean? And i was like clean WHAT?? The MONTHLY?? After THE BEACH?? No, you should not be wearing them there haha, she almost pulled you into a trap
- What cleaning could you use and i said NO CLEANING GUARANTEES getting rid of acanthamoeba cysts not even hydrogen peroxide
- Ok what about RGPs would this px be a suitable candidate? = i said i dont see why not
- Then she said why WOULDNT you recommend the RGP > I said if the cyl comes in soft contact lens design RGPs are way more uncomfortable. Also if cyl isnt THAT high the difference between soft lens and hard lens would be LESS noticeable compared to ppl with surface irregularities and all that jazz so the discomfort may not be as worth it = wouldnt get THAT much more visual gain

- She asked apart from surface irregularities, when would you recommend RGP for toricity? I said if they have high cyls and more sensitive to axis rotations, RGPs may be better, i also think corneal astig is more readily masked in RGPs
- What about ORTHOK?? Is this patient suitable???
    - I said i think -2.75 is on the higher end of suitability for orthok (I think <-2.00 is ideal for orthoK
    - She asked what would you do to assess suitability = topos
    - What would you WANT to see for them to be suitable
    - Ideal candidate for orthoK (from Pauline's lecture):
        - <1.50DC corneal toricity
        - WTR astig
        - Not limbal-to-limbal astig
        - Flat cornea <40D
        - Normal eccentricity (low e aka steep cornea not desirable)
        - Small pupils
            - High refractive error = smaller treatment zone, so big pupils bad VA
        - Not tight eyelids - tight eyelids push lens up or down → decentration

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R -5.00/-2.00 x 45 L: -4.00/-3.00 x 90
*mine was very simple? Because lily has technical issues with cameras not working, unable to unmute/turn on camera LOL LMAO

L: What do you want to know about this patient?

- History symptoms. Motivation, frequency of wear (days/week, hours/day), occupation, VDU. Red itchy dry eyes. Medical history (BP, thyroid, diabetes, etc.), medications. Ocular history of infection, surgery, injuries.

L: For diabetes, thyroid, and BP, can you tell me if these are contraindicated or not for CL wear?

- Diabetes: not contraindicated, have slower corneal regeneration so we need to be mindful and monitor them more frequently if they wear contacts. In case of any epithelial defects.
- Thyroid: can lead to proptosis → inf SPK from exposure keratopathy. Dry eye symptoms, manage expectations (dk if i said if it was contraindicated or not)
- BP: i said inflammation… vasculature changes…??? (i have no freakin clue, she gave me blank looks LOL) NOT contraindicated (beta blockers and diuretics that can be used as antihypertensive drugs can induce dry eye; not contraindicated but precaution ig)

L: What prescription would you give?

- R:-4.75/-2.25x45 (i lowkey got this wrong) L: -3.75/-2.75x90
- Close enough, so should be fine (-4.75/-1.75x45)

L: Can you explain why u chose that cyl for the right eye. Do you need to correct powers for sphere or cyl?

- Correct both sphere and cyl for contact lenses. Usually you can do this by mapping out the meridians and their individual powers then use F(spec)/1-dF(spec) to vertex-correct powers. I was being aware of conventional contact lens brands with -0.75DC steps as most dailies stop at -2.25DC, and I wanted to give them better acuities. (then we went on a tangent and I changed my answer to -1.50DC nah i forgot about 1.75 LOL omg my dum ahh. I was thinking backwards from -2.25DC for the RE)

L: What would type of CLs would you recommend?

- Well it depends on how frequently they want to wear it. Accounting for higher powers, I would choose Coopervision Biofinity toric for RE and Coopervision Biofinity XR toric for LE based on the -2.75DC power on the left eye.

L: Ok, can you tell me more about how you selected this brand?

- SiHy, uses fluorosilicone polymers to transport more oxygen effectively. Fair water content at 48%, Dk/t 160 which is way above the Holden Mertz criteria (Dk/t is actually 116 for the toric Biofinitys) true i forgor that… I was thinking of the normal spherical brand. Base curve is selected by Flat K + 0.8 which can be obtained from corneal topog, and Biofinities are set at ~8.7BC. Modulus is about 0.7 which helps with lens handling.

L: Can you wear this contact lens overnight?

- Yes, it is labelled extended wear so you can sleep in them for 6 nights. I thought biofinity was approved for 29 nights EW?? Not for torics. I think both toric and non-toric are only approved for 6 nights EW?
    - I did some research on this beforehand - yes it does say 29 nights on the table. But its 6 nights of wear - and we want to account for lens hygiene as well so i thought it was impractical to wear 29 nights straight
    - To add onto this: Biofinity USED to be approved for 29nights, as of 03/2024 it got reduced to 6 nights - omg thank you
- I wouldn't recommend it because when you close your eyes you reduce oxygen supply significantly and only rely on the tarsal vasculature. It encourages corneal hypoxia which can lead to stromal changes (oedema and thinning) leading to reduced vision and endothelial cell loss (blebs) which can compromise future corneal metabolism for structural integrity.

L: (cuts off from tech issues like 5 mins) Ok, are these changes permanent?

- Stromal no, endothelium yes. Stromal oedema is temporary as the oedema and lactate accumulation will generally diffuse overtime. Endothelial yes because it can't regenerate and will affect future metabolism (ionic transport)

L: what solution will u choose for this px?

- Optifree Puremoist. MPS solution, two actives (polyquad antibioticmicrobial oops, aldox targets acanthamoeba trophozoite form), disinfect 6hr in storage, rub & rinse before insertion + after removal

L: okk, can i use this solution to clean the lens for acanthamoeba?

- As prev discussed it only targets the trophozoite form. If its cyst form, then its ineffective. It would require additional cleaning methods because MPS cannot clean lenses 100% (i was gonna go into h2o2 but she was ok with this answer xd)

Good job everyone!! And good luck for Wednesday

Wednesday gang pls type up!!

Wednesday

If anyone has questions put them in BLUE - we'd try to answer if u confused on something :)

GOOD LUCK WEDNESDAY - You got this!

Wednesday gang pls type up!!

Guys has no one sat the exam yet? Mine is at arvo, is there even anyone sitting the test rn???not sure:((what time is yours ? 2:30 surely im not gonna be the first right….. Idekk mine is at 3 :((NOOOO im scared, actual shaking :((((same i hate this

:(((

Hi guyssss
My exam is thursday but i have some theories on what she could ask based on the fact that high cyl/mixed astig was the topic yday

MF lenses! Mf vs mono vision
Hydrogel vs SiH
Miserable myopes
Bandage CLs/EW
Signs of hypoxia - link to dk/t
SiH - SEAL, CLPC
RGP - 3+9
Probs digging into MK… (or this is more for thera)

Let us know how it goes, good luck <33

Guys has no one done the exam today 😭

only arvo slots :// sad times



Aww cuute
Guys im so scared D: !!!! same im terrified

HEELOO i did mine
I GOT SMASHED ITS RGP DAY

1. At what occasions are RGP better than SCL ie keractonus, high sph + cyl, ortho-k
   RGPs vs SCLs:

RGP	SCLs
- Easier to clean, less deposits - Lower long-term cost (lenses last for >1 year) - Sharper vision due to tear lens especially irregular corneas e.g. kconus - Not ideal for intermittent wear (hard to get used to) - Sports, swimming - can fall out	- Better comfort - Lower initial cost - Good oxygen transmissibility - May not fit on irregular corneas - Low success with low spherical component and high cyl - a slight change in axis affects VA

2. What the principle behind RGP giving better vision for higher cyl and whats the principle behind for higher sph compare to SCL ?? WHAT DOES THIS MEAN WHAT THE HELL???? like why is rgp giving better vision, then i said for high cyl and sph, then she was like HOW… then i died
   - I guess the tear lens in RGPs neutralises corneal surface so that's why its better for high cyl
   - Higher sph compared to SCL?? I dont understand 😭is this bc the tear lens also introduces additional power ????
   - And maybe the refractive index and thicker lens can correct higher spheres? Is this what she means? lmao
3. If corneal cyl + refractive cyl, how does the lens stabilise on eye - i think she wants the wtheory for bitoric - i said Mandallmore but she was like, no forget that you are not fitting a lens - so i just said prism ballast and tear lens and back surface adjustment… donno if they right or not
4. OD -2.00/-4.00x30 OS -3.00/-4.00 x180
   - Conversion is: R -2.00/-3.50x30, L -3.00/-3.50x180
5. Rgp better or scl better
   - I said scl cuz if they first time wearer, rgp if they prefer higher precision
6. What is the reason that u dont recommended rgp
7. Is rgp easier to wear or scl and why
8. What you gonna recommended if px for sport - DD
9. So what rx u give for DD for this px
   - Could do CV MyDay, max 2.25DC so need to compensate with sph
   - R -2.50/-2.25x30, L -3.50/-2.25x180
10. Why not RGP for sports
    - Can fall out. Also if its occasional use they won't get used to them and cheaper to get SCLs.
11. Any type of RGP you can use for sport - scleral
12. Which one more comfortable - scleral or RGP

- Scleral cuz less lid interaction
- Less movement as well

Then she let me go….. haha…….
Good job!!!!!!!!!!!!!!! You did so well!!!!!!!!!!!!!!! I would've cried ngl thanks for hte info <3 nws hehe
Omg that's so rough :(SOUNDS LIKE U DID GOO THO
I hope soo :(((good luck guys!!!!

Guys IT IS RGP HAHAA we dead

1. So basically why is straight into when r RGP indicated
    - Irregular / high astigmatism
    - Corneal ectasia e.g. keratoconus
    - Orthokeratology e.g. myopia control
    - High sphere more than 20D?
    - Complications from SCLs e.g. SEAL,CLPC
    - Long-term more cost effective
2. Gave rx: -2.00/-4.00 x 30 (-2.00/-3.50 x 30), -3/-4.00 x 180 (-3.00/-3.50 x 180)- asked CL rx and how i got the CL rx (vertex distance)
3. Asked what lens i will give - said different things idk which is right: 1. Biofinitt xr range, or for DD can reduce cyl to common cyl available (ie 2.25 or 2.75) and work out sph equivalent but depends on px and quality of vision they require or can give rgp
4. Soft or rgp - pros and cons to each, what r their complications (3 9 o clock, MK etc)
   SCL complications:
    - SEAL
    - CLPC
    - CLARE
    - Hypoxic changes → neovascularisation

   RGP complications:
    - 3 and 9 o'clock staining - thirsty tear meniscus causes desiccation
        - High edge lift = tear meniscus pulls water from eye by adhesion
        - Low edge lift - minimal tear reservoir to rewet corneal surface
        - Bridging effect - eyelid cannot resurface cornea on blinks
    - Dellen - thinning of stroma
    - Vascularised limbal keratitis - sequelae of 3 and 9 o'clock staining
        - Deposits of connective tissue and vascularisation at limbus
    - Reduced corneal sensitivity threshold
    - Corneal warpage - do not stop CL wear, need to phase withdrawal
        - Gradually reduce wearing time
    - CLPC
5. Cleaning solution for RGP- efficacy (boston advance, hydrogen peroxide)
    - Boston advance - daily cleaner, has conditioning solution as well
        - No need to soak / wait for 6hrs to disinfect
    - Oxysept - H_2O_2 3%, has tablet catalyst
        - Changes colour when neutralised

- Soleko, AOSept Plus - platinum disc to neutralise
- Menicon spray and clean
- Menicare plus multipurpose

This is all my brain rmemebers :((its fried rn
GOOOD joob!! We donskis hahaha

Hello another RGP victim

1. She started off very vague like so why would you choose RGPs over SCLs?
    a. Clarity WHY?--> tear lens , better fitting on high corneal cyl, better rotational stability
2. Why better for high corneal cyl? Cus bst and bitoric have back surface toric design that can align to corneal astig
3. Ok but wont soft cls drape over the cornea and more stable?
    a. No cus they can rock + they cant mould into the irregular corneal shape
4. OK heres rx → gave same rx as above
    a. Same vertex correction but then she asked me to explain so i explained LOL
5. So what lens do you want to give>?
    a. I said it depends on their corneal cyl (maybe i should've considered lifestyle factors here too but my brain was fried) . And we went around in a cycle about RGP fitting on high corneal astigmatic cornea vs SCL. But at the end I think I said id go for RGPs
    b. But she didnt go into super detail regarding SPE&CPE Bitorics and BSTs
6. Name some RGP material?
    a. PMMA → poor O2 permeability, used as trial lens
    b. And then i talked abt first gen second gen and then hyper dk - high O2 permeability
    c. Name one brand? Boston XO (fluorosilicone acrylate)

    1st gen RGPs: silicone acrylates
        - Have siloxane for O2 permeability
        - Poor wettability
        - Ionic charge means lots of deposits and hard to remove

    2nd gen RGPs: fluorosilicone acrylates
        - Better oxygen
        - Lipid deposits still a problem

    3rd gen RGPs - hyper Dk

    Other brand examples: Boston XO2, Menicon Z
7. RGP complications?
    a. 3 and 9 staining, dellen, VLK (she asked me what it stands for lol), steep fit → debris entrapment, flat fit → Fischer thingy I couldnt pronounce it and she smiled .. …
    b. Whats the cause of 3 and 9 o'clock → corneal dessication and i went into detail abt too much edge lift vs too little edge lift
8. RGP solution
    a. I listed aosept, menicare plus, boston advanced, menicon progent

    b. She said which one i would choose - i initially said aosept and then i changed my andwer to menicare plus. And she said why not aosept - i said you have to go through the 6hr neutralisation process which can be inconvenient, risk of toxicity if not neutralised but menicareplus dont need to go through the 6 hrs (i think only a few seconds??? JKS 5mins) + also equally effective against acanthamoeba (trophozoite stage) so i would go for menicareplus and she said ok
9. If this px wants SCL what u use
    a. Biofonity toric XR but adjust the cyl to -3.25 cus it doesnt come in -3.50
    b. And she was like why? And i said cus its the only available one in the market with high cyl
    c. And she was like why not custom made ones? And iwas like from capricornia?? And she was like yeah.. And said its 1) time consuming 2) much higher cost .. 3) more convenient to just give them the available ones like why bother?? And she said okay
    Another example of a SCL for keratoconus is the CLCA Kerasoft - sph goes to +/-30 and cyl up to -15
10. Oh wait she said to list some parameters of biofinity (like she didnt even specify toric/xr toric)
    a. I said its SiH, Comfilcon A, modulus 0.75, dk/t for sph is like 170 something and toric goes down to 116 or something and thats it. I didnt mention power and cyl cus she didnt specify sph/toric/xr toric
11. And she let me go

Ok slay cos i cant do maths LMFAO Gang are we allowed pen and paper? Yess
She literally tells u to write it down and then says the rx in 1 sec
HELPPPP my goldfish brain is not gonna remember AHHA have ur pen and paper ready next to u u will be fine !!!!

3:30pm session: HHAHA I GUESS WEDNESDAY IS RGP DAY

1. She started off the same as well? Why RGP over SCL?
    a. Why RGP better for high Rx? - SCL range is limited, also corneal irregularities. Essentially whatever people have said before.
    b. Also, mentioned that RGP has a less incidence of infection compared to SCL, so ease of care. Lily didn't say anything so I guess it's correct
2. Then she gave me the specs Rx: R -2.00/-4.00x180 L: -4.00/-4.00x180 and asked me to convert.
    a. CL Rx: -2.00/-3.50 x 180; -3.75/-3.50x180
    b. She did ask me how I did the conversion so I said the vertex distance and doing the power cross?
    c. And she asked if there will be a bigger reduction (?) between a -4.00D and -8.00D when converting Rx? So obvs -8.00D, I dont understand what type of question this is?
3. Then she asked what type of CL i will prescribe for this px?

    a. Oh I did mention first depending on what the intended use is: so if sports can go with SCL but if px prioritises vision, RGP
    b. I said can do soft with Proclear or Biofinity Toric XR
    c. But if patient's corneal astigmatism is high, RGP is better because the tear lens can fill in the gap of the steeper meridian to correct the corneal toricity.
4. Then she asked about what solution:
    a. I said H2O2 Aosept because it can be used for both SCL and RGP, it can get rid of acanthamoeba and it is pretty easy to use (just a disc)
    b. She asked in what situation H2O2 is not ideal: when patients have irregular sleep schedule or they are shift workers. H2O2 needs a minimum 6 hours to neutralise
    c. She asked why we cannot use RGP solution for SCL. (I actually didn't know but she kinda helped me????)
        i. She asked if I have seen Boston advanced cleaner and what it looks like LOL
        ii. So the answer is first, these solutions can warp the SCL. Second, the material matrix of SCL can absorb these solutions, leading to solution toxicity.
5. Then she asked about RGP complications:
    a. 3 and 9 o'clock staining, CLPC, SLK, Dellen
    b. And she did ask me what causes 3 and 9 o'clock staining
        i. So too much edge lift leading to thirsty tear meniscus -> dessication
        ii. Too little edge lift leading to not enough tear reservoir
6. And she let me go?? Surprisingly she didn't ask me that many questions. There were so many awkward pauses and I could tell that she was trying to find something to ask LOL. HAHAHA SAME

4:15pm RGP + CL parameters + Disinfectant + RGP complication

1. Rx straightaway: -2.00/-4.00x30, -4.00/-4.00x180
    a. My response: -2.00/-3.50x30, -3.75/–3.50x180
        i. She asks: how did you get these: Vertex Distance, two meridians
2. What lens choice?
    a. SCL or RGP - I said RGP with such high cyl
        i. Why?: better clarity, better fit
            1. How?: tear film created by the back surface design
        ii. Okay here is corneal toricity, it is the same as the refractive cyl (-3.50), what design?
            1. SPE Bitoric
                a. How does the back surface design work? How did you explain this?
                    i. Mandell-moore principal!!
3. Any SCL option?

a. Biofinity XR Toric
    i. What are the parameters: high dk/t, moderate-high modulus, SiHy
    ii. Range for modulus in SiHy?
        1. 0.4~1.5
            a. What is the highest modulus lens
                i. Alcon Airoptix Aqua Night and Day
            b. Why is high modulus beneficial
                i. Easier to control lens for insertion, fits well on the cornea?? (kinda word jumbled here),
                ii. better for physical sports??
                iii. Maybe it could mask some corneal irregularities as well?

4. Disinfectant for RGP
    a. Menicare plus, AOSEPT, Oxysept, Boston Advanced
        i. I chose Menicare Plus,
            1. Why?: easier to use, less waiting time
        ii. Explain how Boston Advanced Works
            1. Abrasive cleaning, surfactant to remove lipid etc
        iii. I mentioned Menicare Progent cleaner
            1. What does it do?
                a. Disinfecting + protein/lipid removal, using every 3~4 weeks
5. RGP brand
    a. Menicon Z, BostonXO/XO2 (hyper DK materials)
    b. PMMA (glass, for trial)

6. RGP Complication
    a. Dellen, 3 and 9 oclock, VLK, Steep fitting with tear debris/hyopixa (know mechanism for all just in case)
    b. Explain difference between Dellen and 3/9 oclock staining

7. “That’s all for today, hope you have a good rest of the afternoon”

4:45PM session

Lowkey I cannot remember anything LMFAO idk how you guys do it

1. **OD -2.00/-4.00x45 OS -4.00/-4.00x135**
    a. Lowkey she just gave me the script and was like :| I was like um? Do you want me to give you the CL Rx LMFAO
    b. OD -2.00/-3.50x45, OS -3.75/-3.50x135 (I kept it at 5 degrees cos the Biofinity lens can have 5 degree increments and I did stress this in future questions to make sure she knew I wasn’t being an idiot)
    c. **How did you get to this Rx?** If it is over +/-4.00 I would correct for vertex distance → split the two meridians up and correct for the vertex distance individually

2. **What lens do you want to give them?**
    a. My strat lowkey be it depends so I can info dump on her and minimise follow-up questions LOL
    b. For soft contact lenses → CooperVision Biofinity XR; only soft contact lens that accommodates for such high cyl and 5 degree increments (lowkey in hindsight I don't think I said XR…shiiiii)
    c. Can also consider RGPs (depending on corneal astigmatism compared to refractive cylinder); didn't go much further than that?? wipes sweat
3. **What would influence you to pick one over the other?**
    a. It depends strat kinda backfired on me here cos I kinda fell into the hole of advantages VS disadvantages and as I went on and Lily kinda cut me off and was like yeah… those are pros VS cons but what would influence you to pick on over the other…shrugging man emoji
    b. RGP
        i. Sharper vision for those with high astigmatism Rx, oblique axes, irregular cornea due to the tear lens being able to compensate for the corneal irregularity + most SCL not accommodating for these powers (I kinda side tracked here and was like yeah Biofinity XR is the only one that does type beat) those who require high visual quality for their day to day work
        ii. Ease of maintenance > than SCL; RGP attracts less lens deposits + better tear exchange → reduced complications
        iii. (this is where I kinda got cut off and asked to talk about more about what influence me to pick on over the other)
    c. Hobbies
        i. Sports → RGP dislodgement risk due to smaller diameter and greater lens movement, would be reduced with SCL given their larger diameter + lesser lens movement
        ii. Swimming → again RGP risk of dislodgement BUT stressed this time Biofinity cannot be used due to the fact that it cannot be disinfected properly → would instead opt for a DDs with a reduced cylinder script + adjusted axis and inform patient of the reduced visual clarity
    d. She did not press on after this… so I assume that was a sufficient answer?
4. **Can you list some RGP materials?**
    a. PMMA
    b. Silicone acrylates
    c. Fluoro-silicone acrylates
    d. Super DK materials (said that this was an improvement of the FSA)
    e. Hyper DK materials (like the Menicon Z)
5. **Can you give me a specific brand?**
    a. Boston XO; Dk=100 for brownie points LOL
6. **What are some properties of RGPs that you look for?**
    a. Oxygen permeability
    b. Wettability
    c. Durability/hardness

    d. Biocompatibility
    e. Then I was like “uh,,,,and yeah,,,,” and she seemed happy with what i said and moved on but I could've also mentioned anterior/posterior edge configuration now that I think about it
7. **What are some RGP solutions?**
    a. Hydrogen Peroxide (AoSept), Multipurpose solution (Menicare Plus), Protein cleaner (Menicon Progent), Boston Advance cleaner + conditioning solution
    b. **What would you select for your patient?** Again I am going for a it depends strategy to cover my bases LOL
        i. Multipurpose solutions: convenience; someone who travels a lot and cannot carry a lot of solutions with them
        ii. H2O2: no preservatives so good for someone who is prone to allergy from the preservatives in MPS solutions BUT require a longer contact time than MPS solutions so wouldn’t recommend for someone with shift work or irregular sleep
    c. **You mentioned Progent earlier - what is it?** Protein cleaner; used once a month to get the protein deposits off
8. **Going back to Biofinity lenses - can you give me some properties?**
    a. I clarified to ask if she meant parameters? She kinda was like shrugging emoji properties/parameters sure sure so I kinda just word vomited at her
    b. 8.7mm/14.6mm/comfilcon A(SiH)/48/116/0.75 modulus/EW 6 nights
    c. I think she was tryna fish for the modulus discussion here
9. **You mentioned the modulus was around 0.75? Is this a high or low modulus?**
    a. I said on the higher end of things ?? LOL
10. **When is a high modulus good and when is a low modulus good?**
    a. High modulus ease of handling for a first time wearer due to its ability to retain its shape well + less prone to breakage BUT higher propensity for mechanical complications (e.g. CLPC, SEAL) → that’s where low modulus comes in
    b. **Follow-up questions of any other reasons low modulus is good (since I kinda just said good things about high modulus)** → started choking here lowkey I said something about comfort; high modulus lenses doesn’t conform to the ocular surface as well as a low modulus lens and can give rise to edge fluting that can induce lens awareness during blink → patient discomfort
11. “That’s all we have time for today, I hope you have a lovely evening!”
    a. You too Lily!! LMFAO

GOOD WORK GUYS!! We survived RGP hell huff huff

I’ll just add she asked me the lens properties of the BOSTON XO or RGP in general and I barely looked into that so I said wetability and DK but she wanted more.

Thursday

If anyone has questions put them in BLUE - we'd try to answer if u confused on something :)

Goodluck everyone =_=" GOOD LUCKK THURSDAY GANGGG

Wait lowkey lemme write stuff to help <333
Slay
Guys im gonna cry if it's not even presbyopia TODAY lols
At least then friday will know <33

Monovision	MFCL
- Less time to adapt, less chair time - Reduced stereopsis and binocularity - Nhnnnnn Greater flexibility and choice with near power an d rx - Distance and near vision not crisp clear - Best for people with CYL and swapping dominance - Low add means better chance of adapting - Good for early presbyopes	- Can get binocularity - May have issues with glare, reduced contrast sensitivity - Best for people with low cyl, can't adapt to monovision, aware vision not as good - Good for late presbyopes

Fitting
Sensory Dominance: +1.50D test (D and N)\
If RE VA reduced with +1.50 @ D = Dominant Eye at distance
If RE VA better with +1.50 @ N = Dominant eye at near

40-44: +0.75 to +1.00
45-50: +1.50
50-60: +2.00
60<: +2.50
(this is just like approx)
Or just match to spec add

Biofinity fitting guide
Low add: CD OU (residual accom can help with near)

Monovision: give it as above^
MFCL: same dominant eye at D and N → no preference for plus → easier to adapt? Can you explain this?i think its cuz u dont need to suppress one eye but like mono u have to
Lowky tho i would just get them to do MFCL anwyays LOL DO IT LOL
I think mono is good for people with low add or high cyl slay
Yah so if the same eye is getting suppressed at D + N (i.e. the non dominant eye) then the patient is going to hate monovision, in that case they have to do mf cls or just like overspecs
I did have a case w eihab but if u wanna know ask me later so i don't confuse u for the oral
OK so monovision we want dom eye to be diff at D & N ?YUP idk this is confusing me 😕oki bb

soooo for monovision one eye is used for D and one eye is used for N - taht's why u needdifferent eyes. If one eye is dom at both D and N then the eye getting suppressed isn't gonna work so you won't see good at the distance of the eye that is not dom
OK SLAY THANK U SO MUCH u saved me <3 nwsssss

Brands Available

- DDs
  - MyDay (no toric): Dk/t 100, BC 8.4, 0.4
  - Oasys 1D Max Dk/t 147, 0.7
  - Oasys 1D Max toric Dk/t 129, 0.7
  - DT1 Dk/t 156 0.7
- Fortnightly
  - Oasys MF with pupil optimised design (no toric)
- Monthlies
  - DT30 (no toric)
  - Bioninfinity: only one that comes in CD (i think);
    - **Bioinfinity Toric D&N: Dk/t116**
      - **+6.50 to +10.00 (-0.50)/+6 to -6 (0.25)/-6 to -10 (0.50)**
      - **-0.75 to -5.75 cyl, 5 deg steps**
      - **Add: +1.00 to +2.50 (0.50)**
- EDOFs
  - Naturalvue: high plus → pinhole effect
  - Seed day 1 → high order aberrations

Wait ill put this withj the MF stuffQUESSSTTTIONNNNNN WHEN DO WE DO OVERSPECS? Is it over normal SVD contacts and they want reading add? Yah

CAMPING;

- DEADASS WILL SIGH IN FRONT OF LILY AND BE LIKE I DO NOT RECOMMEND IT TAKE SPECS
- And if she pushes me then:
- DD if access to clean water and soap, beg patient to wear glasses so i dont have to do this
- EW options: 6 NIGHT FOR EVEYRTING
  - Biofinity - all ranges (e.g. biofinity xr toric, biofinity mf etc.)
  - Jj acuvue oasys 2 weekly
  - Purevision?
- Complications of EW
  - Increased risk of infection!!
    - Epithelial compromise (due to mechanical trauma or trapped deposits, hypoxia related stress, tear stagnation → bacteria proliferates better in moist environment of closed eye → easier to infect IS THIS CORRECT yes
  - Inflammatory
    - CLARE
    - CLPU
  - Hypoxia related
    - Corneal oedema, vacuoles/microcysts,

Layers	Short term Hypoxia	Long Term Hypoxia	Mx

Epithelium	Microcysts (reverse ilum), SPK, vacuoles	Delayed healing Reduced corneal sensitivity	Fit with higher Dk/T lens
Bowmans Layer	Minimal change	haze/scar (chronic oedema)	
Stroma	Oedema, striae	Haze, thinning	
Descemet	folds	Permanent folds	
Endothelium	Bleb (usually immediately after cls insertion)	Polymegathism (size) Pleomorphism (shape) IRREVERSIBLE Due to acidification → cellular stress	Nothing :(it's permanent Bleb temp will go away
Vascular	Limbal hyperamiea + superficial NV (new / reversible; starts at limbus (<1 mm)) clinically significant if >1.5–2 mm from limbus.	Corneal NV (deep indicates more chronic also irreversible?) → can lead to lipid keratopathy → scarring etc	

Follow up for Camping px?

- So consensus is next day HAHA (yeah to see how they react overnight?) and then a week after i guess
- So 1 week trial EW and then see how they are on the 7th day and then week after they can use it for camping ?

When checking for stromal hypoxia what do you use
Anterior OCT? Pachymetry wait idk
SLE for folds and striae
Yeah slit lamp - use high mag and direct illumincation just like push in and out

HOMIES I JUST FINISHED: CONFIRMED PRESBYOPIA:
OMG YAY BLESS TYYYY OMMMGGG
L: Can you describe options for presbyopes with CL:
ME: went through MFCL, Monovision and over the top specs
L: What makes someone suitable for one over the other?
ME: I BLANKED!! But i said I blanked and she moved on quickly and when I remembered I came back to it: someone picky about vision wouldn't like MF and someone who needs stereopsis or is monocular can't have monovision
L: What would you tell/counsel the patient on?
L: Ok so you have a 45 year old with script: RE: -5.00/-2.0x20 LE: -6.00/-1.00x180. What do you do1ME: first went into H&S: what are they using it for, for how long, occupation, hobbies etc Then I'd account for BVD change and pick a lens (I then did that)
L: Ok so based on that script what lens
ME: If full time wear the biofinity toric MF (that script is available). And then use the fitting guide.
L: What if they want to wear 2 times a week
ME: DD is more cost effective- no toric MF option through

- NOTE: I did a little tangent/dug hole here: I said I could offer them a spherical equivalent (which would suck) or glasses over distance contacts forgetting about monovision but she prompted me and I remembered

L: So what lens then?
ME: Coopervision toric (script is available). I'd check for dominance and do full script in dominant eye and add of +1 or +1.25 (depends on spec ADD) in the other eye
L: Do you think this patient would be successful in a spherical equivalent?
ME: No, she has almost -2.00D cyl in the right eye so it wouldn't offer great vision
L: How do you check for dominance:
ME: described the +1.50 test
L: that is all for today

Did you give -4.75/-1.75x20 and -5.50/-0.75x180 with distance centre both OKAY SLAY

- I had -5.50/-1 in left but she didn't press me on that I just immediately explained how I did it
- She also didn't go into picking centre N/D etc- I proactively mentioned the fitting guide and I think she left it there so she could ask about monovision/DD and get to the idea that there are no DD toric MF

9:45am session is complete! (she was really nice surprisingly and froze mid way..)
L: Your patient is presbyopic and wants to consider correction. What are your options
ME: went through MFCL, Monovision and over the top specs
L: What would you recommend for your patient?
Me: It depends on their lifestyle factors and hobbies and occupation, pupil size, tolerance to overall blur. And I went on a big yap about the different concepts of each one so monovision good distance and near but no binocularity (bad for ball sports with gauging distance ce e.g. tennis, golf, bad for surgeons). MFs are not good for big pupils or working in dim conditions due to the simultaneous vision and concentric ring design. Not good for patient who need perfect distance vision
L: You talked about pupil size…. What if they have small pupils…
ME: WAFFLE WAFFLE. If its too small they only look through the optical centre so the retina is not getting the other I, and D ring. You want the pupil big enough for the retina to receive light from the concentric rings

- If the pupil is smaller than 2.5mm, i dont think they are suitable for multifocals
- Ok slay i said 2-3mm ideal

L: Ok but what if the pupil is too big… What is the ideal pupil size you want for your patient then
ME: Dilated means they are looking through multiple concentric zones at once so you would get ghosting and haloes. So the ideal pupil size is medium 2-3mm
L: Ok this is the script. What would you do next RE: -5.00/-2.0x20 LE: -6.00/-1.00x180.
Me: Can we do a distance and near dominance check. May I also confirm their add is between +0.75 and +1.00 as he is early presbyope.
L: Ok lets say he has a +1.00 add, and how would check for sensory dominance?
Me: The sensory dominance check allows us to assess our brain's tolerance to blur for each eye. So at distance we would put +1.50 over both eyes which ever one is less tolerant to blur is the dominant eye at distance
L: Ok but would you do the same for near?
Me: I fumbled here and said we do near dominance over their near add at a near target to eliminate accommodation out of the equation. And then we had a long discussion. Ok so he has a near add of +1.00 and you put +1.50 over the top of that, would it be blurry for him? I said no. So I think she wanted me to say whichever eye has the more benefit from the near add is the near dominant eye.

- ok so how do u do near ? at a near target ??
- It will make BOTH EYES CLEAR bc u are giving a NEAR ADD - its which ever eye is more improved for near - is this at near target and with dist rx Yes
- **If RE VA reduced with +1.50 @ D = Dominant Eye at distance**
- **If RE VA better with +1.50 @ N = Dominant eye at near (do u do it with a near add? Or just distance rx?? Over distance)**

L: Ok so if I put +2.50 over your eye would you find it blurry?
Me: Um no? Just bigger letters.. So I circled back and said I would just do it over their distance refraction with a near target and which eye has better accommodation with the lens is the dominant eye for near.
L: Lets move on and choose a lens and I asked if they wanted monovision or MFs. and she said lets do MFs.
Me: Biofinity toric - went through dk/t spiel, only toric mf available on the market
L: Ok they come back and the vision keeps fluctuating and the lens is not stable on the eye
Me: What do you mean not stable? Is the axis marking disorientated??
L: Let's just say the lens just rotates everywhere and the vision keeps blurring
ME: I would switch them to a monovision with Alcon DT1 and then went through all pros of alcon dt1. -4.75/-1.75x20 and -4.50/-0.75x180. How did y get this because she told me the L eye was near dominant with +1.00 so i just +1.00 over their distance rx. Ohhhhhh
L: screen froze and said thanks, that's all we have.

10:00AM
L: Patient is presbyopic what are your CL correction optionso
L: What would make one suitable over the other
Me: Convenience, visual needs, occupation, dominance eye, tolerating blur
> She kept asking what else
L: How do you assess ocular dominance
+1.50 blur at distance and near - eye least tolerant to blur
L: What do you expect at N

- **Omg i fumbled this one too**
- **Omg yea**

Think she go confused with my answer at near - BUT EYE LESS TOLERANT TO THE BLUR DON'T SAY WHICH ONE BLUR BC NEAR LENSES WILL MAKE THINGS CLEAR
L: Ok, rx is -5.00/-2.00x40 and -6.00/-1.00x180
-4.75/-1.75x40 -5.75/-0.75 x 180
L: How did you calculate the CYL
Me: need to mention split the meridians + formula
L: Ok what lenses would you choose
L: You assess the fit and lens swings from 5 to 7o'clock - what do you do

- SHE ASKED ME THIS AS WELL! I just said scrap this lens and move to monovision and she seemed ok with that answer
- **DITCH THE LENS!!!**

L: You don't want to go with this lens anymore - what can you do?
Me: monovision, distance CL + SVN, spherical MF
L: Patient wants If you were to give monovision
pwhat would you give?
Me: gave her AlconT1 (first one i remembered so just said that) - asked which eye is dominant → near add over distance rx for non-dominant eye

- What is restricted axis? DW I JUST REMEMBERED WRONG :(
- Guys don't forget for mono to go back to the og spec script and recalculate!! D
- don't just take the from the CLs script
- (add is only +1.00 so shouldn't BVD be negligible for the add??
- Nuuu coz 6 is the cutoff for 0.50 reduction if u take it to 5.00 its only 0.25 and that changes the cyl as wellll OHHH
- So first minus +1 from the og script? And then recalculate? Yus WAITT DO U MINUS FOR NEAR WAIT WHAT
- What was the final script you gave?
- I gave R -4.75/-1.75x150 and -4.75/-0.75x40 LE (as spec goes to -5.00/-1.00x40)

Can someone explain this step by step i am confused
Yus ofc! Btw this is the 10:15 and i got basciall,y the exact same and lily didn't questions me so im assuming its okkkkk THANK U

- **Same lily just nodded and said thats ok**

Ok so this is for monovision
R dominant. - keep this the same script
N (for near) - new spec script is -5.00/-1.00x40 (originally -6.00/-1.00x40, given +1.00D add)
So the new cl script is -4.75/-0.75x40 TYYUY

L: Is there any other lenses without axis restriction
Me: CooperVision My Day ^ remembered the axis restrictions wrong my fault lol
L: why did you choose Total1 over MyDay
Me: comfort etc

I think that's all i remember - good luck everyone 😛

Guys my session is at 10:30 but she still hasnt joined
What is going on
What mines at 1045 and im in waiting

She joined at 10:32

10:15
Presbyopic patient
Rx: RE -5.00/-2.00x40 and LE -6.00/-1.00x180
Talked about the three options
& honestly exactly same as 10:00 ^^

10:30 → GUYS IT WAS NOT BAD!!!

45 y/o ADD of +1.00 (she asked me what ADD I expect this Px to have)
L: Talk over the different MF CL options
Me: MF SCL, monovision, distance CL + SVN specs
L: WHy would you choose one over the other?
Me: Fumbled a bit. Talk about advantages of MF SCL (depth perception, better for higher ADDs), not good for those with astigmatism or those who would like crisp vision. Monovision good for younger hyperopes (cheaper, easy to fit, low ADD, better vision BUT binoc vision compromised)
L: gives me the same script as above^^
Hopefully I changed it correctly…
Rx: RE -5.00/-2.00x40 and LE -6.00/-1.00x180
RE -4.75/-1.75x40
LE -5.75/-0.75x180
CAN SOMEONE PLS CHECK AND SEE IF I CONVERTED CORRECTLY - yes thank youu!!
She asked me how I got the cyl. I panicked and just showed her my working out through the camera LMAO - LMAO THIS IS FUNNY HAHA
Then she asked me what I want to do next. I asked her if she wanted me to talk about monovision, she said it's your px
L; how do you check for dominance?
Me: I also got trapped under her near vision dominance question… what answer does she want? I ultimately said the +1.50/+2.00 lens would make it clearer for the Px, but we ask them which eye is worse or not as clear. She didn't seem happy with the answer but just moved on…

- If VA *better* with +1.50 @ N = Dominant eye at near (i think this is the answer she wanted) THANK YOU BUT I THINK I STILL FUMBLED

Ok so then I talked about monovision and what my RE and LE script would be. (Right eye was dominant for distance so -4.75/-1.75x40, left eye was dominant for near so -4.75/-0.75x180)
L: what lens would you choose?
Me: my dumb ass gave 2 options NEVER GIVE 2 OPTIONS, she will dig further
Coopervision Myday at first (i forgot to mention torics "face palm", remembered right at the end)
Then I said Biofinity Toric.
L: But which one would you choose?
Me: said something stupid about dk/t
L: What if i said he wants to wear it 2x /wk (actually was so helpful during this part bless)
Me: talked about how MyDay toric is a daily lens and Biofinity is extended wear (mentioned up to 6 nights/7 days for extra brownie points)
L: Do you really want to give an EW lens to this Px?
Me: confidence crushed, never mind better stick to Myday Toric
L: Why and what are the risks of EW?
Me: corneal hypoxia, MK, CLARE, CLPU, neovasc

L: That's all we have time for 🙂

She was actually so very nice

10:45
GUYS ITS EXACTLY THE SAME
I didnt get any additional questions. I stuffed up the calcs and so many things i said wrong cause i was just blurting out so GOD help BUT ITS DONE
She did ask me actually about what disinfection system id use - i went with h202
Then she said have a lovely rest of your day

And theres nmight and day aqua but thats just spherical
I think XR is the only one that has torics unless im missing something - toric and MF - correct

11:15am chilling same stuff mostly

- Spectacle Rx
  - OD -4.00/-2.00x130
  - OS -6.00/-1.00x180
- First she asked about BVD correction
  - OD -3.75/-1.50x130(wouldnt cyl be -1.75 OD and -0.75 OS??) - the OS right, OD should be -1.75
  - Oppsies, but yes i did speak during it and spoke about how -1.00 not available and would adjust it later as seen below! You wants you to walk her through ur thought process so I just spoke out loud during mine.
  - OS -5.50/-1.00x180
- Then I asked about the px
  - Male, 45, works in office, hasn't worn CLs before, wants to wear CL fulltime to replace glasses, no meds, Hx of ocular disease or infection or dry eye
- Then she asked what cl options
  - Discussed Mono vision, MTF and ruled out distance CL with overspecs since px doesn't want to wear glasses
- Compared mono vs MTF
  - Mono
    - Loss of binocularity
    - More suitable for lower adds in this case
    - Option of DDs reduce risk of CL events (CLPU, infiltrates, MK, CLARE)
    - Often cheaper and can accommodate cyl
  - MTF
    - Has binocularity
    - Risk of glare and halos worse in my px case since they myopic and younger → therefore larger pupil size
    - Only available in SCL toric monthly - increased risk of infection in monthly lens. I also said him being a man (non modifibable risk factor) and first time wearer has risk
  - I ultimately said I do Mono vision first - you mean you do Monovision first? Yes sorry still typing im bad typier may take me a bit to type and fix errors!
- Asked me what lens

- Picked Alcon DT1 toric
- She said Distance dom OD and near dom OS - I said this was good and improve chance of adaptation since alternating dom
    - OD -3.75/-1.25x130
    - OS (redid initial power cross) -4.75/-0.75x180
        - For add I said he could be between 1.00 or 1.50 but I would base on specs add. For this I said I would go with 1.00 since he is a office worker and would likely have a desktop
        - I also did go through my thoughts about whether monovision would be sutiable for extended computer wear since only has the one eye → fatigue. Said could consider MTF if this would be unsuccessful since both eyes have sensory input.
- She said why Alcon
    - High Dk/t 140, spoke about water gradient technology and delificon A material (30% core and 100 outer surface), Moderate Modulus 0.7 - reduced risk of mechanical events like SEAL and CLPC (seen in first gen SiH)
    - Also said since daily disposable less of a risk of infective complications explained each and spoke up gram positive and negative events. Inflammatory vs infectious
    - Spoke about DD vs monthlies, we spent alot of time here but she was happy with my answer. I found that since I covered all my bases and explained everything I could she asked me little questions and ws happy. Thankfully didnt get any of her WHAT ELSE, so she seemed to like that I went into everything without needing her to probe me.
- She asked if lens was unstable e g axis kept changing what would i do?
    - I said since its not just axis misalignment but rather a poor fit I would scrap the lens
        - SCL limited BC and parameters so would have to pick new lens
- Then she asked about MTF
    - I picked Binifinity (Not XR)
    - Actually I fear we didnt even get to discuss this that was all lowkey
    - I did mention MTF lens eg Capricornia?what this vs Bio with CV has better dk/t 115 vs 60ish
        - For SCL toric mtf you only have option of Sih Coopervision biofinity vs Capricorna(monthly or EW) lens they are both made to order but biofinity has better Dk/t. They are both monthly only CV has 6 nights approved EW. But she didn't ask me for two options I just mentioned both and said why I went with Biofinity!
    - Overall she was in a good mood and very nice :)

11:30
But legit same thing as the morning session
L: She gave me -4.00/-2.00x130, -6.00/-1.00x180
A: R: -3.75/-1.75x130, L:-5.50/-1.00x180
She asked how i got this and i said VD conversion
Said BF MF torics
L: what if lens fluctuates from 5 to 7 o'clock
A:: ditch the lens change to monovision
L: ok so what would u choose
A: I said Alcon Dailies lololol idk for Monovision
RE dis Dom

LE near dom
I forgot to back to the original rx and change it :,)
Should be
R:-3.75/-1.75x130
L:-4.50/-1.00x180 I THINK
L: why is DDs better than monthlies
A: Omg bruh i mind blanked when she asked why is DDs better than monthies

- Said less chance of CLPC, less chance of infection risk, more convenient LIKE WHAT ELSE
- Don't have to clean it as much?
- She kept saying what else i legit forgot

12pm all finished!!

- Rx R +4.00/-2.00x130, L +6.00/-1.00x180, patient is interested in CLs, what do you want to do?
    - How old is the patient? 45
    - Occupation? Officeworker
    - Why do you want to wear CLs? For work everyday and to wear to the beach
    - Have you worn CLs before? No
    - Hobbies, allergies, general health, medications, inj/surg/infection etc
- What do you do now?
    - Explain options for CLs i.e. overspecs, monovision, MF
    - Slit lamp exam check staining evert lids
    - Sensory dominance
- How do you do sensory dominance?
    - +1.50D for distance and near
- What lens would you recommend?
    - Because they are an officeworker and monovision only gives you N+D i choose MF so they have intermediate range
    - CV biofinity MF toric monthly - but DO NOT wear to beach
    - Converted Rx and follow fitting guide for +1 Add
- Then what do you do?
    - Wait 10-15 minutes and check VA binocularly, not behind phoropter
    - Check slit lamp - tightness, orientation, lag, movement on blink
- If it’s moving from 5 to 7 o’clock what do you do?
    - That’s way too much, have to swap to a different lens
    - Since there is no other MF that will deal with this cyl we have to do monovision
    - I would do CooperVision MyDay monovision
- How would you check their vision
    - Binocularly, not behind a phoropter
    - Add +/-0.25 trial lenses to whatever eye needs adjustment e.g. reading or distance
- Why wouldn’t you choose a monthly lens?
    - Oh facts yeah I should do a monthly lens if they’re wearing it every day it would be more economical for them (NOT TO THE BEACH THOUGH)
- Yeah sure but why would you choose a daily lens over a monthly lens
    - Oh like in general?
    - Less risk of infection, convenience of not having to do cleaning solution
- What are some CL-related complications?
    - CLPC, SEAL, CLPU, SICS, IK, CLARE

    - Hypoxia - I started to get into microcysts and oedema then she asked for non-hypoxic related complications lmao
        - I said so not neovasc? HAHAHAAH
- Of these, which are more likely to happen in monthlies vs daily disposables?
    - SICS - no solutions in DDs
    - CLARE - only in EW
    - Then she basically made me describe what the cause of each of the complications is e.g. inflammatory vs mechanical
- Ok i think that's all we have time for :)))))

ARVO

CL script: +4.00/-3.00 x 130 LE +6.00/-2.00x 180

L: What do you want to do before fitting this patient?
ME: considerations - history and symptom - age, occuption, environment, systemic health (diabetes, thyroid, immunocompromised), allergies, hobbies
L: Convert the Rx
Me: I completely fked it but it should be
+4.25/-3.25 x 130
+6.50/-2.25x180
L: What contacts do you want to use
XR for Right, Biofinity, not XR for LE since the cyl is in rage -L: Why Biofinity? yeah
Me: Good Dk/t, initial comfort is good
L: How would you go about telling the patient about wearing the lenses?
Me: Initially I said wear them for 1 week
L: How many days in the week
Me: everyday? Then backtracked to 3-4 times a week for 8 hours (she said 'what if i told you this patient was a first timer')
L: What if the patient is uncomfortable in the first 5 hours?
Me: Then take the lenses off and rewear them the day after
L: Why would you do this?
Me: Let them adapt to the contact lenses since they are a first timer. So get them to wear it for 2-3 hours and see how they go
L: And what if they are comfortable with that?
Me: To slowly increase the wear time until they are fully comfortable
L: What would you tell the patient if they want to sleep in the contacts
Me: No, increase risk in bacterial infection since bacteria grows best under moist closed environments
L: What can happen if you sleep in the contact lens besides risk of infection?
Me: Hypoxia, increase in deposits
L: What can increase in deposits lead to?
Me: CLARE, CLPU
L: What solution would you use
L: What do you need to do with optiFree Puremoist? **(rub and rinse??, rub 20 sec each side of CL, rise each side for 10 secs)**
Me: i had no clue what she meant - i didnt even remember about rub and rinse fmd
L: Do you pour the solution over the contact lens in the case or pour the solution first in the

case?
Me: i said pour solution then contact lens in and then pour it over to the mark
L: then she asked me again what action would you want to do with MPS?
I just said not sure
L: ok thats all we can do today - kms

02pm
Okay same as before with some extra.
L: What lens?
L: What do you know about the Biofinity range of CLs?
L: What would you do if the px insisted on EW?
L: What are some complications of EW? (inflammatory, infectious, hypoxia)
L: Explain how these complications are more common in EW
- I said CLPC (more contact time with lids and lens + build up of deposits), CLPU (more build up of exotoxins)

L: What hypoxia signs would you see? When would they be more evident? (had no clue for when lmao)
L: When would you review a EW patient? - I had no clue lmao
L: okay gonna leave it there

2:30pm

L.Given Script: +4.00/-3.00x130 OD, +6.00/-2.00x180
Q.What do you first do?
A. Patient History
Q. And then what?
A. Refraction, Slitlamp and then lens selection
Q. What CLs conversion and lens chosen?
A. +4.25/-3.25x130, +6.25/-2.25x180
Q. lens chosen?
A. Biofintiy Toric XR for OD and Biofinity Toric OS
Q. What to do if lens is rocking between 5 and 6 oclock?
A. Change lens - steepen BC
Q. What other lens would you choose?
A. Acuvus oasys because 8.4mm BC but do give -2.25DC and the rest spherical equivalent
Q. What would you do after giving the lens to the patient?
A. Acuvue Oasys 1 day for astigmatism
Q. What is the spherical and cyl range of Acuvue Oasys 1 day?
Q.What cleaning solution recommended?
A. Optifree
Q. What makes one H2O2 system different from another? Why is Oxysept only usable in SCLs?

A. Aosept vs Optifree
Finished

2:45PM

LILS: +4.00/-3.00X130 +6.00/-2.00x180, patient is interested in CLs how do you want to get started?
M: ask about reasons for wanting CLs, age, occupation, how often they want to wear it, previous experience in wearing CLs, hobbies, occupation, infections, surgeries… yes… based off that I would selection a lens.
L: patient wants to wear lenses to replace their glasses, they want to wear it everyday, havent worn CLs before, no hobbies, works in a office.
M: okay i will probably start them off on….biofinitiy torics? … Hang on can i convert the lenses to CL Rx first?
L: yep and laughs
M: +4.25/-3.25x130 R (biofinity XR toric) +6.50/-2.25x180 (biofinity toric)
L: why did you choose those lenses?
M: they want to wear the CL everyday so monthly lenses will be better financially in the long term + -3.25CYL is available in XR torics.
L: okay you have those lenses and you fit it and it rotates 6 to 7 o'clock what do you do?
M: is this on the right eye or both eyes?
L: both eyes.
M: apply LARS rule; and add or subtract based on …
L: but what would you add?
M: uh (caught me off guard said random number LOL); uh 10 degrees?
L: why 10 ?
M: oh sorry each hour is 30 degrees so 10 degrees? Oh wait sorry 30 degrees LOL
L: laughs ok so you got 30 degrees.
M: oh wait actually thats too much of a rotation so i would change the lens
L: ok… to what lens
M: MyDay Torics
L: but why? Why are you changing to a daily lens now?
M: im just trying to change the lens to assess another lens fit on the eye to see if rotation is still large.
L: yes but why did you choose daily?
M: true, i guess i will pick a fortnightly then.
L: ok what lens?
M: (digging myself into a hole). If they want to continue wearing it everyday then maybe a fortnightly lens.
L: ok what lens?
M: acuvue moist for astigmatism?
L: is that fortnightly?
M: oh wait sorry i meant acuvue oasys for astigmatism.
L: silence… okay why not another monthly lens.
M: okay i guess we can do another monthly lens.
L: ok can you give me another example?
M: um Acuvue VITA lenses?
L: ok and is there another one?
M: UMM ULTRA?? For astigmatism by B+L

L: okay do you know another one?
M: biofinity XR toric instead of just torics? LOL
L: laughs ok what solution do you want to use?
M: OptiFree PureMoist; it's MPS so it will increase compliance due to simplicity (can rinse, disinfect with one bottle)
L: how do you instruct the patient to clean the lens?
M: wash + dry hands, rub and rinse on both sides of lens prior to insertion and storage in case?
L: anything else?
M: not to top up solution; always dry face down.
L: anything else?
M: um fill the solution to the line on the CL case?
L: what about the case besides upside down?
M: replace every 1-3M or everytime you finish the puremoist solution?
L: so acuvue moist? Is this a lens they can sleep in?
M: no?
L: ok what about ULTRA?
M: no? I think its only the Biotrue (for 6 nights and 7 days) and i think the airOptix night and day one?? (idk what i was thinking or saying for this)
L: what type of lens is Biotrue one?
M: hydrogel?
L: so what's the Dk like?
M: its lower on a hydrogel so but idk what the value is
L: so do you think it will be approved for extended wear?
M: no
L: do you know the modality?
M: no
L: Well it's not approved for EW. ok what else did you say can you tell me about airoptix? Is it approved for EW?
M: um im not really sure of the actual name but im pretty sure theres a day and night one that's approved.
L: ok whats the process to start someone on EW?
M: check corneal health + predispositions + let them wear for 1-2 nights and do a review (lily nodding) to assess for signs of hypoxia + try to extend this period out more.
L: is there something you'd recommend to patients to do for EW?
M: UM? Not swim in it?
L: no no no the question is, is EW something you would recommend patients?
M: oh no no i wouldnt personally recommend it
L: yeah why not
M: um it decreases their mentally when it comes to compliance, they will be at a higher risk of infections, inflammatory responses.
L: is there any other reasons?
M: increased risk of infections, inflammatory reponses, hypoxia?
L: what are the inflammatory responses you're thinking about
M: CLARE, hyperaemia, neovascularisation, CLPU?

L: what is the general reason why neovascularsaiotn develops after EW?
M: the cornea is avascular structure and in EW there is hypoxia so limits to amount of oxygen + needs to create new blood vessels to supply oxygen???
L: what are other signs of hypoxia?
M: microcysts, vacuoles, stromal oedema, polymegathism, pleomorphism
L: what do they look like?
M: microvysts and vacuoles on epithelium (reversed and unreversed illumination), stromal oedema (striae and folds - wrinkles and fine lines), you can see it on slit lamp but changes in cell size and shape for poly and pleo
L: you can't see it on slitlamp at all??
M: um maybe its harder to see it on slit lmap?
L: how do you see it on slit lamp?
M: hm i actually dont think you can see it on slit lamp, i think you need a special type instrument? awkward laugh
L: okay what's this special type of instrument called
M: im not sure
L: ok that's all we have time for

THIS IS CRAZY… its the detail for me. Sounds like you slayed it tbh well done

330
WTFFFF bitoric specific features

1. Same rx **+4.00/-3.00X130 +6.00/-2.00x180 -** no questions asked i asked about age 38 yo officeworker first time CL but wants to wear it all the time so i convert correctly and suggest biofinity XR OD biofinity toric OS - she pulls the rocking question and asks me alternatives and how to fix it (rocking between 5 and 6) - DONT SAY LARS RULE
2. I was stumped for a bit then asked about VA comfort etc she says good question but VA bad - i was stumped bc only toric lens with large cyl
   Kinda went in circles but came to giving total30 toric with SE OD (got the SE part wrong but she was nice said how did u get that? Slow down its ok then fixed my mistake)

3. Then she asked what would i do RGP fit for this px i said i need their topo and i will discuss different toric options etc she said if Cornea cyl = refractive cyl - said SPE bitoric she said good then the grilling starts lol
4. Do u expect rotation in a SPE bitoric? Tldr: no bc 2 powers 2 curved specified so expect it to be perfect for px cornea also even if it does rotate bc Cornea cyl = refractive cyl i mentioned SPE is free to rotate without impacting vision - she smiled and was happy FINALLY
5. What is the stabilisaiton system used for bitorics? I said prism ballast she seemed fine with it but idk lols

Excellent job
Fin

3:45 :
L: Gives script: +4.00/-3.00x130, +6.50/-2.25x180. What do you want to do?
M: Px history, health conditions, frequency of wear, sport, hobbies, why they want to wear the lenses?? First time wearer?
L: Ok so this px is a first-time wearer and wants to try contact lenses, no health conditions what do you do?
M: um let me… work out the script LOL (and still gets it wrong but she saved me HAHA)

- Script: RE +4.25/-3.25x130 LE: +6.50/-2.25x180

L: Ok so what do you want to do now?
M: Biofinity XR RE coz thats the only one that comes in that power and Biofinity for LE
L: Ok so what are you looking for?
M: So im looking at the lens fit, centration, movement, tightness, orientation of lens marking
L: What is adequate movement?
M: 0.2mm - 0.5mm
L: what is adequate tightness?
M: 40-60%?? But 50% ideal??
L: ok the lens marking swings between 5 and 6 oclock
M: … so its not stable…
L: yes
M: then it means its not an adequate fit so i would consider changing it
L: ok so what would you change it to??
M: wel, i would consider changing this to a DD since the only reason why i went monthly in the first place was bc it was the only one avail in that cyl (biofinity XR), dailies have less chance for deposit build up and hypoxic effects etc and also easier for a first time wearer to teach daiy lenses so they can discard it after
L: ok… what would you choose then ….
M: My day toric but i would adjust for spherical equiv for RE
L: script?
M: RE: +3.25/-2.25x130 LE: same (+6.50/-2.25x180)
L: ok but why did you choose dailies?? Why not monthlies??
M: (didn't i already say this….) and im like oh well easier for first time wearer?? And we wanna see if this fits better ??
L: yeh but what else?? Can you tell me why you would choose a monthly over daily for this px
M: I guess cost (Lily smiles), so if they are wearing every day then monthlies are more economical but i would discuss this with the px first and ask them if cost is an issue or not and decide from there. But first pref is DD!
L: ok wel if u were to go monthlies what other options are there
M: Total 30 → comes up to -2.25 cyl as well … and i would do the same sph equiv
L: ok moving on… what solution would you give this px
M: Optifree
L: WHICH OPTIFREE?
M: oh.. Optifree pure moist?
L: why?

M: because out of all the MPS this covers for acanthomoeba trophizite form coz its got aldox compared to other solutions which have biguanides
L: why not H2O2
M: never said you couldnt, H2O2 would be better to remove protein deposits but risks of toxicity if they don't neutrialise it right so we need to make sure we educate the px well about the process
L: how do you teach the px about using MPS solution
M: Rub and rinse the lens on both sides for about 10-20 minutes, fill CL case up with solution and put it in and tightly seal and leave for 6 hours.
L: What about case hygiene
M: rinse case with MPS and rub and rinse again for 10-20 seconds and then dry with clean tissue and leave face down to air dry away from bathroom
L: yeh but what else do you need to tell px about the case???
M: discard the case after 3 months
L: ok thats all we have time for have a lovely rest of your day! You too!

4:00 shes in good mood HAHAHA
L: Gave script R: +4.00/-3.00x130 L: +6.00/-2.00x180, what lens will u choose?
M: Need more information. Age? Motivation? Frequency…
L: replace glasses 38yo
M: converted to CL, then say Biofinity XR for RE and Biofinity toric for LE
L: What is the CL rx? Why these lenses?
M: R: +4.25/-3.25X 130, L: +6.50/-2.25X180, bc Biofinity XR is the only available lens for RE script
L: What to check in SLE?
M: centration, lens movement, lens lag, tightness, markings at 6 oclock
L: What if rocking between 5 and 6 o'clock in both eyes?
M: Poor fit, switch to other lenses, but for the RE rx Biofinity XR is the only available SCL option.. then i say try fitting RGP
L: What if the px doesnt like the idea of RGP?
M: Apply LARS rule, subtract by 30 degrees in both eyes
L: Do u think it will be clear after refitting?
M: No bc the lens keeps rotating
L: Then what can u do?
M: I will do Alcon Total 30 for astigmatism, but for the RE the vision will be compromised. Then cal spherical equivalent for RE +3.75/-2.25X130 IDEK is that right?
L: back to RGP, what lens design?
M: need more information, do topography to know K values, corneal astig etc, if refractive cyl similar to corneal cyl = SPE bitoric, if not =CPE bitoric
L: Wt SPE and CPE stand for?
M: Spherical sth and Cylinder sth… (i have no idea…)
L: how to determine the BOZR for bitoric designs?
M: Flatten the meridian using Mandell moore

L: By how much?
M: im not sure…
L: Wt solution to clean?
M: Menicare plus MPS
L: why not H2O2?
M: MPS is easier to use, and H2O2 need a strict 6-hour neutralization period
L: Thats all

4:15 she did not smile or laugh with me 😕
L: Rx +4.00/-3.00x130 +6.00/-2.00x180, what u gonna do
M: Hx, what they wanna use the lenses for, occupation, environment, sports, hobbies, previous CL wear, Hx of injuries surgeries infections, MHx: diabetes, thyroid dysfunction, Medications: antihistamines, beta blockers, antipsychotics, hormone replacement therapy
L: okay so 38 year old office worker wants to wear these CL everyday
M: okay give me a second im just gonna convert the Rx ⇒ +4.25/-3.25 x 130 | +6.50/-2.25 x 180
L: okay good, now what lens would you give them
M: Lets go with biofinity torics, and it'll have to be the extended range in the eye with the -3.25 Rx.
L: okay and now what are you gonna look for on the fit
M: centration, movement, lag, tightness, orientation of toric marking
L: where should the toric marking be?
M: ideally at 6 o'clock
L: okay now what if i told you it was at 5 o'clock
M: I would have to compensate with the LARS rule depending on which eye the rotation is happening in
L: rotated to 5 o'clock in both eyes
M: okay so i would go to +4.25/-3.25 x **100** | +6.50/-2.25 x **30**
L: can you explain how you got to this? (giving me a chance to correct myself)
M: i broke it down step by step and realised i was meant to SUBTRACT cause it's swinging to the RIGHT and quickly corrected myself ⇒ +4.25/-3.25 x **100** | +6.50/-2.25 x **150**
and she was happy with that
L: okay now what solution would you like to give to this px
M: given that they are a 1st time wearer, i would give them a MPS like opti-free puremoist
L: why opti-free puremoist?
M: it has Aldox which covers acanthamoeba in its trophozoite form whereas other MPS do not
L: so does Opti-Free puremoist cover for acanthamoeba?
M: only in its trophozoite form, not in its cystic form
L: okay now what if i told you this px likes to go to the beach

M: i'd advise against swimming in the lenses due to risk of infection esp. with EW lenses. And to be careful about sand getting in the eye and getting trapped behind the lens. But if they must, i would give them DDs for beach days and tell them to discard the lens afterwards. And I would also advise against swimming, but if they must, they should wear goggles to minimise risk of infection. And after swimming, wait at least 15-20 mins before lens removal or irrigate with saline or lubricants before removing the lens due to lens adherence caused by the hypotonic water.

L: and what lens and Rx would you give them?

I would go for Alcon Total1 Dailies, and the Rx would have to be modified in the eye with -3.25DC because they only come up to -2.25DC. So i would go spherical equivalent so +3.25/-2.25 x 130.

L: can you explain how you did this? (she's giving me a second chance to correct myself)

M: i broke it down step by step and realised i mathed it wrong so i corrected myself ⇒ +3.75/-2.25 x 130.

L: good, and what axis would you order for this patient? (remember he had to the 5 o'clock rotation, she tryna trick me here)

M: i would stick with the original Rx and order the axis of 130 because 1s have a different stabilization method to Biofinities and just because there was a rotation on the biofinities doesn't mean that there would be one on the Alcon Total 1s.

L: *nodding in agreement* yes good. Now you mentioned earlier that you wanted to ask about diabetes and thyroid disease? Why is that?

M: theres a few reasons, so diabetics have delayed corneal healing, so given any staining or abrasions, they'd be susceptible to infection for a longer time period. Diabetics also have reduced corneal sensation, so they may not feel FB trapped behind lens or other complications. Diabetics also have a fragile basement membrane which makes them more susceptible to abrasions or erosions of the epithelium. And for thyroid disease, hyperthyroid disease AKA graves disease causes proptosis which can lead to exposure keratopathy and dryness, whilst hyPOthyroidism can affect lacrimal gland production and again, lead to dryness

L: so what how does that affect your lens selection

M: in that instance i would go for a lens with lower water content as they'll be less likely to draw moisture from the tear film and cause dryness of the ocular surface.

L: Okay that's all we have time for today, i hope you have a lovely evening (says this with no smile)

M: *smiling ear to ear* thanks lily you too, cya (im awkwardly trying to figure out how to end the call)

4:30

GUYS I think I cracked the code (pls take it with a grain of salt…)

I basically explained in detail as to what I will do and what I would discuss with Px and I felt like I barely got probed from lily (either that or its the end of the day and she wants to leave early)

- *Basically I went into it remembering that we should have a practitioner's mindset and it always depends on Px's needs, so basically just saying it depends as opposed to going straight into prescribing a lens*

L: Rx R +4.25/-3.25x130, what questions do you ask initially for Px
M: Age, Motivation, how many times you would wear CL, hrs, general health, meds, allergies, etc…
L: Px comes in wanting CLs 2x a week for soccer, what would you prescribe
M: Can I convert first?
L: Yes
M: +4.50/-3.50x130 L +7.00/-2.50x180 (I said this is what i converted my script into, BUT because cyls dont come in those, the script would actually be +4.50/-3.25x130 L +7.00/-2.25x180)
M: (I just kept speaking to justify my thought process, I think that might be what Lily wants) I would first tell the patient that a daily lens is recommended as he is just wearing it 2x/week however, as it doesn't come in his prescription, we might have to get him a spherical equivalent that will not give as crisp of a vision. Or what I can do if he plays for say a championship team or doing this as a career, is recommend something like a monthly biofinity toric XR lens.
L: Well as an optometrist, what would you recommend?
M: I think if its just twice a week as casual sport, I will recommend a DD as its more cost-effective and efficient
L: What will be your new DD script?
M: R +4.00/-2.25x130 L +7.00/-2.25x180
L: So did you change it from your CL or Rx prescription
M: (Me thinking I got it wrong) Sorry I would convert from spec Rx
L: so why would you convert from spec Rx
M: (Me realising you can't really convert from spec Rx) sorry I think I misunderstood the question, I would convert from CL Rx
L: Can you just tell me the whole process of how you would do it
M: I will vertex distance correct Px first, then apply the spherical equivalent to the CL Rx (Idk if this is right but she moved on)
L: Apart from cost, what are some benefits of DD's over monthlies
M: Less likely for protein/deposits, inflammation and infection like CLPU and CLARE
L: Are you saying that CLPU is infectious?
M: No, it is **sterile** and inflammatory reaction, just like CLARE
L: Would RGP's be good for this Px?
M: No as it's for sport and likely to fall out of the eye and its expensive
L: Is it more expensive than DD?
M: It's more expensive considering you are wearing it for 2x/week
L: Why else would you not want to wear RGP's apart from it falling out
M: Because it may cause more trauma as soccer is contact sport? (wasn't sure)
L: (Guiding me back to the answer), think about why he is wearing it
M: Ohhh, as he is wearing it only for twice a week, it is more difficult for him to adapt to the rigid lens as adaptation normally requires consistent wear
L: Okay, I think that's all, I think I can let you go

Key takeaway from mine, just explain in detail until lily cuts you off, and keep thinking about the case as a practitioner as opposed to just wanting to prescribe stuff

5:00
L: A patient with script R +4.00/-2.00x30 L +5.00/-2.00x180 comes to you wanting CL. What do you do?
M: Start by asking what they want it for. Frequency of wear? Occupation? Hobbies? Existing dry eye symptoms? Any allergies? Injuries, surgeries, infections? Medical health and medications?
L: Px wants for wearing every day to replace glasses. Office worker. No hobbies. No to everything else.
M: Recommended Biofinity Toric lens since they are wearing every day so this is most cost-effective option for them
L: What power do you prescribe?
M: I said R +4.00/-2.25x30 but should've been +4.25/-2.25x30, L +5.25/-2.25x180.
L: Why did you choose these powers?
M: I applied the VD correction formula
L: How?
M: For powers of >4D, need to apply the formula
L: What is the normal BVD?
M: 12mm
L: What if the BVD was 20mm? What power do you prescribe?
M: I blanked lol
L: …
M: …
(long awkward pause)
L: Do you expect the CL power to be higher or lower?
M: Higher
L: By how much?
M: Panicked and said by 1D
L: Uhh
Then went on a whole tangent about BVD and VD correction and asked a bunch of qs - if this power then what correction, etc.
L: Why did you choose Biofinity Toric lens?
M: Is a monthly lens, is toric, high dk so good for eye health
L: What other properties does Biofinity have?
M: SiHy material
L: What else?
M: Moderate water content - good balance between comfortable and not causing too much ocular dehydration
L: What else?
M: Mid modulus of elasticity - easy to handle but not too stiff that it will cause complications
L: What kind of complications?
M: CLPC

L: What else?
M: idk
L: Why is good oxygen permeability needed?
M: Cornea is avascular so needs oxygen from the environment. Without oxygen it cannot metabolise and it will get damaged and then cells will die.
L: What would you see if this happens?
M: Corneal neovascularisation
L: What else?
M: Microcysts, corneal haze
L: What else?
M: idk
L: What would you see in the endothelium? What would it look like?
M: Hazy
L: How do you look at the endothelium?
M: Optic section. Instead of the usual slight roughness it would be a lot more rough. Also specular reflection
L: Okay so what would you see?
M: Roughness?? And opaque??
L: What else?
M: idk
L: Okay so that's all we have time for today.
M: internally dying

Friday

If anyone has questions put them in BLUE - we'd try to answer if u confused on something :)

Hey friday gang anyone in morning session? Im in 9:30 - ooh good luck! Pls update us on what she ends up asking bc who knows atp T-T

Hi I am cooked
I dont even know nvm

09:30 - Zoom muted me in midway lol

1. **Give me some examples of EW**
    a. AirOptix night and day, airoptix hydraglude, biofinity
2. **How to determine if the patient is suitable for EW**
    a. Age, Ocular health, allergy, general health
    b. Forgot to mention meds lol
3. **Is Age a specific thing for EW**
    a. I said not really but i think maybe can predict compliance???
4. **Complications of EW**
    a. Higher risk of infection, inflammation, hypoxia
    b. What else? - idk
5. Rx R -3.00/-3.00x20 L -2.50/-2.50x67, convert it
    a. -3.00/-2.25x20, -2.50/-2.25x67
6. **How did you get the left eye cyl?**
    a. Explained vertex distance, I am not sure if she is talking about the axis so I just said -2.50/-2.25x70 at the end. She didn't ask more so idk
7. **What lens you giving him? Let say he is going to Fiji for a week**. I though she is saying about EW so I said biofinity torics.
8. **When are you reviewing him?**
    a. One day to check compliance and then one week
    b. One day or one week? I honestly don't know so i said why not both
9. **No there is water in Fiji he is in a resort, he just don't want to clean the lens.**
    a. (me dumb brain thought he is going camping) - NO EW THEN I will give him DD.
10. **Forget about DD, How do you minimize the risk of infection for EW, he is also a first time wearer.**
    a. In office I&R teach
    b. Educate no water contact
    c. Educate signs of infection, inflammation, go to find a optom/ophthal/ER ASAP
    d. Take spare specs with him
    e. What else? Silence
    f. Lubricants? IDK
11. That's all for the day I have to go bye

Good luck! Thank u so much!! U did good
What lens did you pick for this px?? - I said biofinity toric cuz i thought she was talking about EW, but then i realised she was talking about DD. I correct myself I will do DD instead but she

didn't ask what I am giving and just keep asking me about EW lol. Lol yeah it might be due to the time so she cant delve into another set

10:00am - same as above

1. Asked about what EW lens options there are?
2. How to determine if someone is suitable for EW?
3. What do you need to tell someone that wants EW?
4. Complications of EW?
5. Script R -3.00/-3.00x20 L -2.50/-2.50x67 (same as above)?
6. What lens are you giving if someone (first time wearer) goes to Fiji for a week?
7. What other options are there?
8. Thats all we have time for!!

She was pretty nice today

10:15am: EAXCTLY THE SAME AS ABOVE EVEN THE SAME SCRIPT

- Mention that EW is good for shift workers, emergency personnel, old ppl with poor dexterity
- She will ask for review schedule
    - First time CI wearer - wear as if normal monthly lens for first week with gradual buildup of hours
    - Then after a week can try wearing overnight + review the next day
    - Then review again after wearing as EW for 1 week before they leave to FIJI
- She didnt ask about swimming but i mentioned to stay clear of water and bring backup specs
- Also asked about what are the risks of EW - same as above

10:30AM - lily having so many technical issues i started at 10:40 ish (she was very chill and nice)

EXACTLY THE SAME SCENARIO AS ABOVE literally no changes

- Due to time limit, didn't ask me about water but I mentioned it anyway + staying away from soil/vegetable matter if going hiking/camping there
- Give examples of EW lenses available
- Asked for risks of EW → I went into complications → asked me further about inflammatory complications since I mentioned it
- How to determine if px suitable for EW
- Reasons why we wouldn't give EW
- Reasons why we WOULD give EW
- Exact same Rx R -3.00/-3.00x20 L -2.50/-2.50x67
- How would you start the px on EW?
    - Kind of vague, you can ask her to clarify → i went into in-office demonstrations of I&R, cleaning techniques, hand washing techniques, educated on adverse events, then review schedule ^^ same as above :))

10:45 she went into extended wear first but then diverted into if the patient with script below wanted to wear the lenses for every day adn they are first lensn wearer. Bruh i prepped my answers for extended wear and was all ready for it but she went into the scenario yest :(((

1. Give examples of EW lenses available
2. Asked for risks of EW
3. Gave script OD: -4.00/-2.00x45 OS: -6.00/-2.00x135
4. Converted it
5. If they wanna wear the lenses everyday what would you give → talked abt DD first then monthlies more cost effective
6. What do u want to see on slit lamp? Fit- centration, movement, lag, tightness - forgot toric lens marking but she prompted
7. What is ideal tightness? I said 50-60%? Shes like 50%? Im like maybe closer to 60% L: so closer to the tight side why? M: better stability with the toric lenses
8. Asked if it was rotated 5 oclock? What would you do? I said to change lens brand to another stabilitsation technique.
- Which lens? What stabilisation
9. Thats all. Have a good rest of your day

11:45
1.+6.00/-2.00x 12, +7.00/-2.00 x 45-> converted
2. DD lens, solution, 4x week, first time
3. H&S, Sihy vs H, modulus compare, monthly-> what lens?
4. Review period, SLE check what